جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : همت مصطفى شعبان خليل
المحافظة : الجيزه **الحي / المدينة :** الصف **العنوان :** مركز الصف الجيزه
رقم البطاقة : 25812082101321 **رقم التليفون :** 01003590026 **تاريخ تسجيل الإستمارة :** 20-10-2026
العمر(تاريخ الميلاد) : 08-12-1958 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: همت مصطفي شعبان خليل
National ID: 25812082101321

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	**الوقت :** -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية

بطاقة تحقيق الشخصية



همت

مصطفى شعبان خليل

٢٦٢ د-هضبة الاهرام

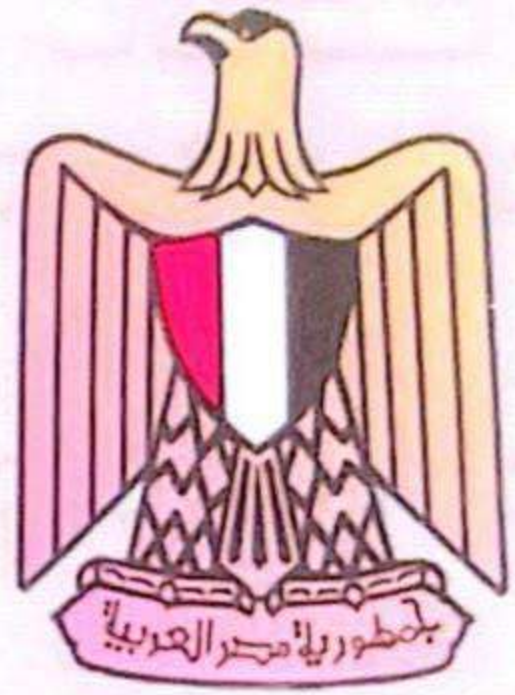
الهرم - الجيزه

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LN4838537

National ID – Front
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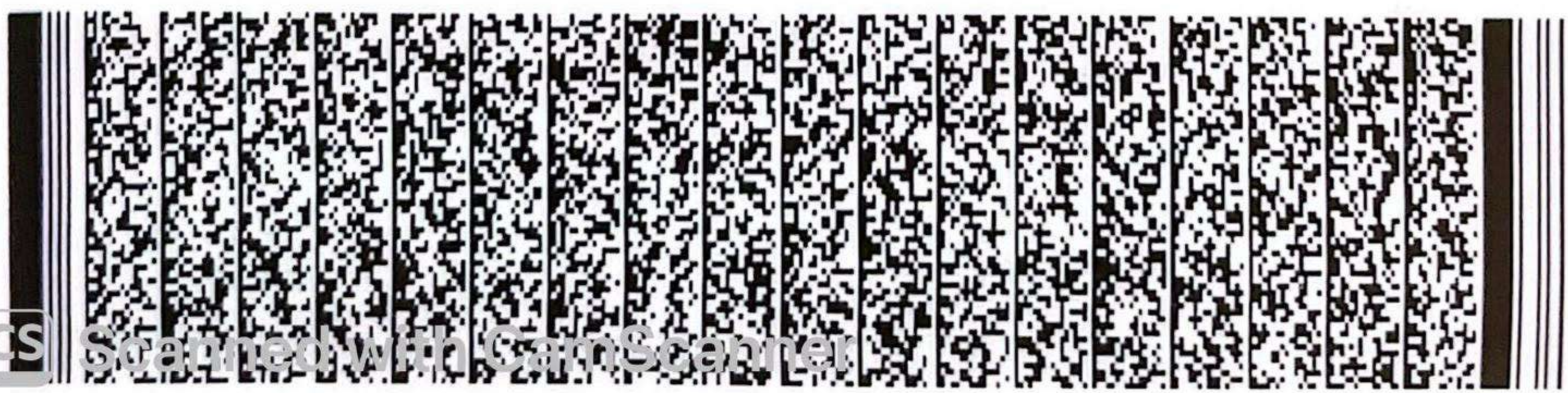


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صيدلانية حرة

أنثى مسلمة أرملة

١ البطاقة سارية حتى ٢٠٣٢/١٢/١٠



National ID – Back
[[DOCTYPE ID_BACK]]

Scanned with CamScanner

مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

08/05/2026

MDT FORM

Name:	Hemat Moustafa Shaban .	MRN:	688
Age:	68 Y	ID:	25812082101321
Menopausal Sta	POST	Phone:	
NRS SCORE:		Marital Status:	
BMI:		Family History:	NO
CO Morbidity:	HTN	Reason Of MDT:	MTS ...DP ...NEW LINE

Clinical Data:

BX: BIL IDC G2

ER: 8/8 BOTH PR: RT 8/8 LT 5/8 HER2: -VE BIL KI67:

R L



TNM

Sonomammography:

Mets Workup:

History:

A CASE OF METASTATIC BIL. BREAST CANCER TO BONE FROM THE START (2022) PT RECEIVED AI (ARAMIDEX A'ND ZOMETA) TILL 7/2025 WHEN PT DEVELOPED ABNORMAL BOWEL HABITS THEN ABSOLUTE CONSTIPATION THEN IN 8/2025 PT UNDERWENT RT HEMICOLECTOMY
PATHOLOGY MALIGNANT ROUND CELL INFILTRATING INTESTINE WITH METASTATIC LOBULAR CARCINOMA
SO PT WAS SHIFTED TO AROMASINE
SLIDE REVISION AT OUR CENTER SHOWED METS. ILC
IHC HR 8/8 HER2 NOT AMPILIFIED BY SISH
MDT DECISION IN 11/2025 WAS TO START IBRANCE AND FASLODEX
BUT PT DIDN`T RECEIVE ANY TTT TILL NOW
LAST CT CAP 7/2026 DP AS REGARD BONE
PT STARTED FASLODEX AT BAHEYA C1D15 10/8/2026

Recommendations:

PLEASE FOR IBRANCE WITH FASLODEX

MDT Outcome & Plan

For FAslodex- Ibrance

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

MDT Form
[[DOCTYPE:MDT_FORM]]

GUSTAVE/ ROUSSY INTERNATIONAL

Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي انترناشونال – مصر معمل الباثولوجي

جوستاف روسي

Patient Name	هدى مصطفى شعبان	Case No.	343431
Sex/Age	Female 67 Y	Patient No.	25074
Registered Date	21/7/2026 03:10:44 PM	Referred By Dr.	Out Patient
Validation Date	03/8/2026 02:28:48 PM	Branch	Gustave Roussy
Reporting Date	08/8/2026 03:03:14 PM	Pathology No.	SR-340/26

Breast Slide Revision

Clinical Data: Left breast 9 o'clock mass, BIRADs 5.

Nature of Specimen: Referred slides (Tru-cut biopsy).

Gross Examination:
- Four referred slides including 1 (H&E) stained slide and 3 immunostained slides for (ER, PR & Her2) labelled by (2088/26).

Microscopic Examination:
* **Histological type:** Invasive lobular carcinoma.
* **Grade:** 2 (differentiation 3, nuclear pleomorphism 2, mitosis 1).
* **Lobular carcinoma in situ:** Absent.
* **Ductal carcinoma in situ:** Absent.
* **Angioinvasion:** Absent.
* **Perineural invasion:** Absent.

IPT:
- ER: Strong nuclear reaction in 98% of tumor cells (score 8/8).
- PR: Negative nuclear reaction in tumor cells (score 0/8).
- Her2: Faint incomplete membrane reaction in more than 10% of tumor cells (score 1+).

Diagnosis:
Left breast mass, Referred slides (Tru-cut biopsy):

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY INTERNATIONAL

Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي فترناشونال – مصر معمل الباثولوجي

جوستاف روسي

Patient Name	هنت مصطفى شعبان	Case No.	343431
Sex/Age	Female 67 Y	Patient No.	25074
Registered Date	21/7/2026 03:10:44 PM	Referred By Dr.	Out Patient
Validation Date	03/8/2026 02:28:48 PM	Branch	Gustave Roussy
Reporting Date	08/8/2026 03:03:14 PM	Pathology No.	SR-840/26

- Invasive lobular carcinoma, Grade 2.

IPT:

- ER: Positive (8/8).
- PR: Negative (0/8).
- Her2: Low (score 1+).

* ***Scoring system: "ER, PR"**

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%
and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

* ****Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

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Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي انترناشونل – مصر معمل الباثولوجي

جوستاف روسي

Patient Name	هنت مصطفى شعبان	Case No.	343431
Sex/Age	Female 67 Y	Patient No.	25074
Registered Date	21/7/2026 03:10:44 PM	Referred By Dr.	Out Patient
Validation Date	03/8/2026 02:28:48 PM	Branch	Gustave Roussy
Reporting Date	08/8/2026 03:03:14 PM	Pathology No.	SR-340/26

tumor cells.

Signature
Dr. Walaa Ibrahim Ghonaim, MD

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

MRN:	2022198204	D.O.B:	08/12/1958
Patient Name:	HEMT MOSTAFA SHABAN KHALIL	Sex:	Female
IC No.:	25812082101321		
Location:	External Clinic		

CYTOPATHOLOGY

(Cytology - 1 Site) فحص خلوي

Specimen Id	2606180231	**Collection Date**	18/06/2026 02:09 PM
Accession No	CYTO-26-530	**Received Date**	18/06/2026 02:10 PM
Sample Type	Fluid/ Smear	**Doctor**	Shahenda Mohamed Laythy Fahmy

Nature of Specimen:
Pathologic Left axillary lymph node

Gross Picture:
Four smears (pap stained) and cell block

Microscopic Picture:
Smears: Atypical epithelioid cells in hemorrhagic partially air dried background.
Cell block: Scanty

Diagnosis:
Left axillary lymph node, U/S guided FNA cytology:
-Atypical epithelioid cells, **FOR TISSUE BIOPSY.**

Comments:

Category Code (LN)
C3: Atypia: atypical cells present, lymphoid or other of uncertain nature and

Signature
Esraa Esmat, MD

Reported and electronically endorsed by
Esraa esmat mohamed abdelmonem mohamed

Pathology lab director:
Dr. Hazem Abdelazim, MD, IFCAP

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Pathology Report
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MRN:	2022198204	D.O.B:	08/12/1958
Patient Name:	HEMT MOSTAFA SHABAN KHALIL	Sex:	Female
IC No.:	25812082101321		
Location:	External Clinic		

SURGICAL PATHOLOGY

Biopsy Small - (1 Site)

Specimen Id	2606180230	**Collection Date**	18/06/2026 02:09 PM
Accession No	SUR-26-2088	**Received Date**	18/06/2026 02:09 PM
Sample Type	Tissue	**Doctor**	Shahenda Mohamed Laythy Fahmy

Clinical Information
Lt breast 9 o'clock highly suspicious lesion (BIRADS 5).

Gross Picture
Tissue cores 1-1.5 cm

Microscopic Picture
A case of invasive lobular carcinoma, grade 2. No in-situ component nor lymphovascular emboli were detected. Stroma is desmoplastic with mild inflammatory cell reaction.

Diagnostic Summary

Tumour type:
Invasive lobular carcinoma

Laterality
Left

Specimen type:
US-guided biopsy

Location
9 o'clock

Histological grade

Mitosis score (1-3):
2

Tubular formation score (1-3):
3

Nuclear grade (1-3):
2

Histological grade (1-3):

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٨ شارع علوية بن شارع الهرم بجوار نادي الجزيرة
مستشفى بهية الشيخ زايد
Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

2

Peritumoural lymphovascular invasion
Absent

Diagnosis

Lt breast (9 o'clock), US-guided biopsy :
Invasive lobular carcinoma, grade 2.

Comments
ER, PR and HER2 immunostaining is ordered.

Biopsy reporting category
B5b= Invasive cancer

Signature
Dr. Hend Elghareeb

Co-signed by:
Dr. Al Shimaa Zakaria, MD

Reported and electronically endorsed by
El Shimaa Zakaria Fathy Ali

Pathology lab director:
Dr. Hazem Abdelazim, MD, IFCAP

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مستشفى بهية الشيخ زايد
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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

- The liver is mildly enlarged with homogenous CT texture. No focal lesions or intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogeneously enhancing portal vein and is main branches as well as its main tributaries.
- Still noted bilateral adrenal glands well defined soft tissue nodules measuring about 1.1 cm in each side likely adenomas.
- Normal appearance of the pancreas, spleen, kidneys, aorta , IVC and the pelvic organs.
- Under- filling of the urinary bladder with no definite filling defects or diverticular outpouching.
- No pathologically enlarged abdominal or pelvic lymph nodes.
- No abdominal or pelvic masses or collections.
- Uterine fundus well defined lesions are still seen
- **The revised bone window revealed :**

1. **Size and number progression with further sclerosis of the wide spread mixed lytic and sclerotic osseous lesions involving the axial and appendicular skeleton.**
2. L4 and L5 grade I degenerative spondylolisthesis
3. Dorsolumbar spondylodegenerative changes
4. No CT evidence of vertebral collapse.

Opinion:

- ***Currently noted:***

1. ***Left breast retroareolar ill defined lesion.***
2. ***Right minimal pleural effusion with mild right pleural thickening.***

- ***Progressive course regarding:***

1. ***Left breast UIQ heterogenous enhancing parenchyma.***
2. ***Left axillary level I enlarged lymph nodes.***
3. ***Left supraclavicular small lymph nodes.***
4. ***Some of the right axillary level I prominent lymph nodes.***
5. ***Few small prominent mediastinal lymph nodes***

- ***Status post right hemicolectomy showing operative bed distortion and mild colonic mural thickening likely post operative changes with resolution of the previously seen small bowel obstruction.***
- ***Size and number progression with further sclerosis of the wide spread mixed lytic and sclerotic osseous lesions.***
- ***Otherwise, Rather stationary course since the previous study.***

Reported By,
Dr. Ali Saber, MSc
Dr. Mennatallah Mohamed Hanafy,MD **08-Jul-2026 03:21 PM**

CT Scan
[[DOCTYPE:CT_SCAN]]

PATIENT NAME: HEMT MOSTAFA SHABAN KHALIL **GENDER:** Female **DATE :** 07-Jul-2026 10:49 AM	**PATIENT ID:** 2022198204 **AGE:** 67 y **ORDER:** 3911941,3911942

POST- CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS

Compared to the previous study dated 02-07-2025, the current study shows:

1-CHEST

- **Currently noted:**

1. Left breast retroareolar ill defined lesion measuring 21 x 17 mm.
2. Right minimal pleural effusion with mild right pleural thickening reaching 9 mm at right lower lobe medial segment with underlying subsegmental atelectasis.

- **Progressive course regarding:**

1. Left breast UIQ heterogenous enhancing parenchyma.
2. Left axillary level I enlarged lymph nodes, the largest measuring about 1.7 x 2.4 cm compared to 1.1 x 2.2 cm surrounded by stranding of the fat planes.
3. Left supraclavicular small lymph nodes the largest measuring 8 mm compared to 5 mm previously.
4. Some of the right axillary level I prominent lymph nodes the largest measuring 12 x 6 mm compared to 8 x 4 mm.
5. Few small prominent mediastinal lymph nodes largest at subcarinal measuring 13 x 9 mm compared to subcentimetric previously.

- **Still seen** few bilateral pulmonary nodules, the largest at right upper lobe posterior segment measuring about 7 mm.
- No evidence of left pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- No pericardial effusion detected.

2- ABDOMEN AND PELVIS

- **Status post operative showing operative bed distortion and mild colonic mural thickening likely post operative changes with resolution of the previously seen small bowel obstruction.**
- **Still noted:**

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CT Scan
[[DOCTYPE:CT_SCAN]]